

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male** Sample Type : Serum
Phone : Registered On : 31-12-2025 10:45
Ref.Dr. : Doctor C Collected On : 31-12-2025 10:46
Req.No : **LAB122522087** - 40003750775 Received On : 31-12-2025 13:25
Report Status : **Final** Reported On : 31-12-2025 19:00

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Uric Acid, Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Uric Acid	: 6.20	mg/dL	3.5-7.2

Method : Uricase PAP(Phenyl AminoPhenazone)

Interpretation

Increased :

renal failure, gout, leukemia, polycythemia, and hemolytic anemia. decreased renal clearance due to drugs like thiazides

and furosemide, the use of drugs like barbiturates, some patients with alcoholism, high protein weight-reduction diet.

Decreased :

Use of uricosuric drugs like probenecid, Allopurinol, cortisone, estrogens, Fanconi syndrome, etc.

Reference : Beckman coulter kit insert

Note :- Suggested Clinical Correlation *

End Of Report

Dr.Vijay Babu B,MBBS.MD
Consultant Biochemistry

Dr.Sowmya Gayatri .C
M.B.B.S MD
Consultant Biochemistry

Dr.S.A.Nabi Ph.D
Sr.Biochemist

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male**
Phone :
Ref.Dr. : Doctor C
Req.No : **LAB122522087** - 40003750775
Report Status : Final

Sample Type : Serum
Registered On : 31-12-2025 10:45
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Received On : 31-12-2025 13:25
Reported On : 31-12-2025 18:50

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Kidney Function Test Profile - Doctor c

Blood Urea : 20.39 mg/dL 17-43

Method : GLDH Kinetic

Interpretation: -**Increased In**

Impaired kidney function, congestive heart failure, salt and water depletion due to vomiting, diarrhea urinary tract obstruction, hemorrhage into GI tract.

Decreased In

Malnutrition, pregnancy, infancy, acromegaly, low protein- High carbohydrate diet, impaired absorption, some patients of nephrotic syndrome.

Blood Urea Nitrogen : 9.53 mg/dL 7.0-18.0

Method : Calculated

Creatinine,Serum : 1.01 mg/dL 0.67-1.17

Method : Modified Jaffe Kinetic

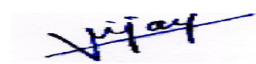
BUN Creatinine Ratio : 9.44

Method : Calculated

Urea Creatinine Ratio : 20.19

Note :- Suggested Clinical Correlation *

****End Of Report****


Dr.Vijay Babu B,MBBS.MD
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Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male**
Phone :
Ref.Dr. : Doctor C
Req.No : **LAB122522087** - 40003507073
Report Status : **Final**

Sample Type : Urine
Registered On : 31-12-2025 10:45
Collected On : 31-12-2025 10:46
Received On : 31-12-2025 14:40
Reported On : 31-12-2025 18:29

DEPARTMENT OF CLINICAL PATHOLOGY

Complete Urine Examination (17)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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PHYSICAL EXAMINATION:

Colour	: Pale Yellow		STRAW TO YELLOW
Appearance	: Clear		
Reaction (pH)	: Acidic 6.0		5.0-8.0
<i>Method : Methyl Red & Bromothymol Blue</i>			
Specific Gravity	: 1.020		1.000-1.030
<i>Method : pKA Change</i>			
Protein / Albumin	: Negative		NEGATIVE
<i>Method : Tetrabromophenol blue</i>			
Glucose / Sugar	: Negative		NEGATIVE
<i>Method : Glucose oxidase/Peroxidase</i>			
Blood	: Negative		NEGATIVE
<i>Method : Peroxidase</i>			
Ketones	: Negative		NEGATIVE
<i>Method : Sodium Nitroprusside</i>			
Bilirubin	: Negative		NEGATIVE
<i>Method : Dichloroanilinediazonium</i>			
Leucocytes	: Negative		NEGATIVE
<i>Method : 3 hydroxy5 phenylpyrrole + diazonium</i>			
Nitrites	: Negative		NEGATIVE
<i>Method : Diazonium + 1,2,3,4 tetrahydrobenzo (h) quinolin 3-ol</i>			
Urobilinogen	: 0.2	mg/dL	0.2-1.0
<i>Method : Dim ethyl aminobenzaldehyde</i>			

MICROSCOPIC EXAMINATION:

Pus Cells	: 2-3	/HPF	0-5
Epithelial Cells	: 1-2	/HPF	0-5
RBC	: Absent	/HPF	Absent
Casts	: Absent		Absent
Crystals / Amorphous deposit	: Absent		Absent
Others	: Nil		Nil

Method: Reflectance Photometry, Automated Urine Analyzer/Microscopy.

Reference:

Print Date : 31-12-2025 18:39

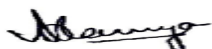
Page 1 of 2

Client	: Simplify Wellness India(Doctor C) - C		
Patient Name	: Mr.HAJIMALUNG BABA		
Age / Gender	: 31 Y(s) / Male	Sample Type	: Urine
Phone	:	Registered On	: 31-12-2025 10:45
Ref.Dr.	: Doctor C	Collected On	: 31-12-2025 10:46
Req.No	: LAB122522087 -	Received On	: 31-12-2025 14:40
Report Status	: Final	Reported On	: 31-12-2025 18:29

Siemens Multistix 10 SG Kit insert. Textbook of Medical Laboratory Technology B. Godkar,3rd Edition.

Note :- Suggested Clinical Correlation *

****End Of Report****



Dr.A.Ramya Priyadarshini
Consultant Pathologist

Dr.Gaurav Rastogi
MD,Pathology
Senior Consultant

Dr.Sushmitha Reddy A
Consultant Pathologist

Dr.K Kiran
Consultant Pathologist

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male**
Phone :
Ref.Dr. : Doctor C
Req.No : **LAB122522087** - 40003750794
Report Status : Final

Sample Type : **WHOLE BLOOD EDTA**
Registered On : 31-12-2025 10:45
Collected On : 31-12-2025 10:46
Received On : 31-12-2025 14:32
Reported On : 31-12-2025 16:58

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Glycosylated Hb (HbA1C)-Doctor C

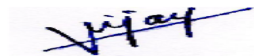
Glycosylated Hb (HbA1C) : 5.8 %
Method : High-performance liquid chromatography
Normal:<5.7
Prediabetes:5.7-6.4
Diabetes:>6.5

Estimated Average Glucose : 119.76 mg/dL
Method : Calculated

Glycosylated Hemoglobin : 39.89 mmol/mol
Method : Calculated

Note :- Suggested Clinical Correlation *

****End Of Report****



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Dr.S.A.Nabi Ph.D
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Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male** Sample Type : **WHOLE BLOOD EDTA**
Phone : Registered On : 31-12-2025 10:45
Ref.Dr. : Doctor C Collected On : 31-12-2025 10:46
Req.No : **LAB122522087** - 40003750794 Received On : 31-12-2025 14:32
Report Status : **Final** Reported On : 31-12-2025 17:13

DEPARTMENT OF HAEMATOLOGY

Haemogram

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Hemoglobin <i>Method : Non-Cyanide Photometric Measurement</i>	: 16.3	g/dL	13.0-17.0
HCT/Haematocrit <i>Method : Calculated</i>	: 50.1	%	40.0-50.0
RBC Count <i>Method : Electrical Impedance</i>	: 6.05	millions/cumm	4.5-5.5
MCV <i>Method : Calculated</i>	: 82.8	fL	83.0-101.0
MCH <i>Method : Calculated</i>	: 27.0	pg	27.0- 32.0
MCHC <i>Method : Calculated</i>	: 32.6	g/dL	31.5-34.5
RDW-CV <i>Method : Calculated</i>	: 14.6	%	11.6-14.0
RDW-SD <i>Method : Calculated</i>	: 45.5	fL	39-46
Platelet Count <i>Method : Electrical Impedance</i>	: 4.18	lakhs/cmm	1.50-4.10
MPV <i>Method : Calculated</i>	: 8.8	fL	7.40-10.40
Total WBC Count <i>Method : Electrical Impedance</i>	: 8760	Cells/cmm	4000-11000

Differential count**(Method:VCS/Leishman Stain/Microscopy)**

Neutrophils	: 67	%	40-80
Lymphocytes	: 28	%	20-40
Monocytes	: 02	%	2-10
Eosinophils	: 03	%	1-6
Basophils	: 00	%	0-1

Absolute Leucocyte Count

Absolute Neutrophil Count <i>Method : Calculated</i>	: 5869.20	Cells/cumm	2000-7000
Absolute Lymphocyte Count <i>Method : Calculated</i>	: 2452.80	Cells/cumm	1000-3000

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Req.No : **LAB122522087** - 40003750794
Report Status : **Final**

Sample Type : **WHOLE BLOOD EDTA**
Registered On : 31-12-2025 10:45
Collected On : 31-12-2025 10:46
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Reported On : 31-12-2025 17:13

Absolute Eosinophil Count	: 262.80	Cells/cumm	20-500
<i>Method : Calculated</i>			
Absolute Monocyte Count	: 175.20	Cells/cumm	200-1000
<i>Method : Calculated</i>			
Absolute Basophils	: 0.0		
<i>Method : Calculated</i>			

Reference :Dacie and Lewis Practical Hematology,12th Edition

Note : These results are generated by a fully automated hematology analyzer and the differential count is done on a peripheral smear.

Method:

Fully automated haematology analyzer (Beckman Coulter DxH 900) (Photometric Measurement, Electrical Impedance, VCS Technology, Leishman's Stain and Microscopy)

Note :- Suggested Clinical Correlation *

****End Of Report****

Gaurav Rastogi

Dr.Gaurav Rastogi MD,Pathology
Senior Consultant

Dr.Sushmitha Reddy A
Consultant Pathologist

Dr.K Kiran
Consultant Pathologist

Dr.A.Ramya Priyadarshini
Consultant Pathologist

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male**
Phone :
Ref.Dr. : Doctor C
Req.No : **LAB122522087** - 40003750775
Report Status : Final

Sample Type : Serum
Registered On : 31-12-2025 10:45
Collected On : 31-12-2025 10:46
Received On : 31-12-2025 13:25
Reported On : 31-12-2025 20:18

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Thyroid Stimulating Hormone (TSH)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
TSH <i>Method : CLIA</i>	: 2.54	µIU/mL	Children: Birth-4d:1.0-39.0 2-20wk:1.7-9.1 21wk-20y:0.7-6.4 Adults: 21-54y:0.4-4.2 55-87y:0.5-8.9

Interpretation:

- 1.T3 &T4 values may be altered due to changes in serum proteins,pregnancy,drugs,nephrosis etc.In such cases Free T3 and Free T4 may give more appropriate thyroid status.T3 levels fluctuate rapidly to stress and non thyroid illness.
- 2.TSH values may be transiently altered in fever,severe infections,liver disease,renal and heart failure,severe burns,trauma and surgery.
- 3.Drugs that decrease TSH values include L-DOPA,Glucocorticoids,Heparin.Drugs that increase TSH include-Iodine,Lithium,Amiodarone.
- 4.TSH exhibits diurnal rythm,peaks at 2.00- 4.00AM and has lowest level at 5.00-6.00PM

Reference:

Beckman caliper study of pediatric Reference values.Beckman kit insert,Teitz Fundamentals of clinical chemistry.

Note :- Suggested Clinical Correlation *

****End Of Report****

Dr.Vijay Babu B,MBBS.MD
Consultant Biochemistry

Dr.Sowmya Gayatri .C
M.B.B.S MD
Consultant Biochemistry

Dr.S.A.Nabi Ph.D
Sr.Biochemist

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male**
Phone :
Ref.Dr. : Doctor C
Req.No : **LAB122522087** - 40003750775
Report Status : Final

Sample Type : Serum
Registered On : 31-12-2025 10:45
Collected On : 31-12-2025 10:46
Received On : 31-12-2025 13:25
Reported On : 31-12-2025 18:54

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Lipid Profile

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Total Cholesterol <i>Method : CHOD-POD</i>	: 226.0	mg/dL	Desirable: < 200 Borderline: 200 – 239 High: >= 240
HDL Cholesterol <i>Method : Enzymatic Immunoinhibition</i>	: 40.1	mg/dL	Major risk factor for heart disease: <40 Negative risk factor for heart disease: >60
LDL Cholesterol <i>Method : Calculation</i>	: 153.35	mg/dL	Optimal : < 100 Near Optimal : 100–129 Borderline High : 130-159 High : 160–189 Very Highy : > 190
VLDL <i>Method : Calculated</i>	: 32.55	mg/dL	6-38
Triglycerides <i>Method : Glycerol Phosphate Oxidase (GPO), Peroxidase (POD)</i>	: 162.76	mg/dL	Normal: < 150 Borderline High: 150 - 199 High: 200 - 499 Very High: >= 500
CHOL / HDL Ratio <i>Method : Calculation</i>	: 5.64		3.5-5.0
Non - HDL Cholesterol <i>Method : Calculated</i>	: 185.90	mg/dL	<130
LDL/HDL Ratio <i>Method : Calculated</i>	: 3.82	mg/dL	1.3-3.5

Reference:The National Cholesterol Education Program Adult Treatment Panel III (NCEP-ATP III) Guidelines.

Note :- Suggested Clinical Correlation *

End Of Report

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Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male** Sample Type : Plasma - F
Phone : Registered On : 31-12-2025 10:45
Ref.Dr. : Doctor C Collected On : 31-12-2025 10:46
Req.No : **LAB122522087** - 40003751110 Received On : 31-12-2025 13:16
Report Status : **Final** Reported On : 31-12-2025 16:21

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Glucose, Fasting (FBS)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Fasting Plasma Glucose	: 93	mg/dL	Normal : 70 - 100 Prediabetes :100-125 Diabetic : >=126
Method : Hexokinase			

Interpretation:

Criteria for diagnosis of Diabetes mellitus.

FPG $\geq$ 126 mg/dL, Fasting is defined as no caloric intake for atleast 8 hours.

Remarks:

Fasting hypoglycemia may be observed in persons taking certain diabetes medications, antibiotics and alcoholic beverages. Additionally intestinal disorders, endocrine disorders, insulin surge, nature of diet, reduced food intake than usual and stress are among several other factors. Please correlate clinically.

Reference:American Diabetes association guidelines 2021.

Note :- Suggested Clinical Correlation *

****End Of Report****

Dr.Vijay Babu B,MBBS.MD
Consultant Biochemistry

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Dr.S.A.Nabi Ph.D
Sr.Biochemist

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male** Sample Type : Serum
Phone : Registered On : 31-12-2025 10:45
Ref.Dr. : Doctor C Collected On : 31-12-2025 10:46
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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Calcium, Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Calcium, Serum	: 10.3	mg/dL	8.8-10.6

Method : Arsenazo III

Interpretation

Increased :

Increased levels may be seen in hyperparathyroidism, vitamin D intoxication, milk-alkali syndrome, thiazide diuretic use, and granulomatous diseases

Decreased :

Hypoparathyroidism, chronic renal disease with uremia and phosphate retention, malabsorption of calcium and vitamin D, and insufficient calcium, phosphorus, and vitamin D intake.

Reference:

Beckman kit Insert.

Note :- Suggested Clinical Correlation *

End Of Report

Dr.Vijay Babu B,MBBS.MD
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Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male**
Phone :
Ref.Dr. : Doctor C
Req.No : **LAB122522087** - 40003750775
Report Status : **Final**

Sample Type : Serum
Registered On : 31-12-2025 10:45
Collected On : 31-12-2025 10:46
Received On : 31-12-2025 13:25
Reported On : 31-12-2025 18:59

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Liver Function Test (LFT) - Doctor c

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Total Bilirubin <i>Method : DPD</i>	: 0.77	mg/dL	0.3-1.2
Direct Bilirubin <i>Method : DPD</i>	: 0.15	mg/dL	<0.2
Indirect Bilirubin <i>Method : Calculated</i>	: 0.62	mg/dL	0.0-0.9
SGPT / ALT <i>Method : IFCC without Pyridoxal Phosphate</i>	: 46.62	U/L	0-50
AST/SGOT <i>Method : IFCC without Pyridoxal Phosphate</i>	: 38.54	U/L	0-50
Alkaline Phosphatase <i>Method : IFCC AMP-Buffer</i>	: 110.78	IU/L	30-120
Total Protein (TP) <i>Method : Biuret</i>	: 8.04	g/dL	6.6-8.3
Albumin <i>Method : Bromocresol Green(BCG)</i>	: 4.41	g/dL	3.5-5.2
Globulin <i>Method : Calculation</i>	: 3.6	g/dL	1.8-3.8
A/G Ratio <i>Method : Calculated</i>	: 1.23		0.9-1.8
AST/ALT Ratio <i>Method : Calculated</i>	: 0.83		1.0

Note :- Suggested Clinical Correlation *

End Of Report

Dr.Vijay Babu B,MBBS.MD
Consultant Biochemistry

Dr.Sowmya Gayatri .C
M.B.B.S MD
Consultant Biochemistry

Dr.S.A.Nabi Ph.D
Sr.Biochemist

Client : Simplify Wellness India(Doctor C) - C
Patient Name : **Mr.HAJIMALUNG BABA**
Age / Gender : **31 Y(s) / Male** Sample Type : Serum
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Report Status : **Final** Reported On : 31-12-2025 18:50

DEPARTMENT OF CLINICAL BIOCHEMISTRY

Blood Urea Nitrogen (BUN)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Blood Urea	: 20.39	mg/dL	17-43

Method : GLDH Kinetic

Interpretation: -**Increased In**

Impaired kidney function, congestive heart failure, salt and water depletion due to vomiting, diarrhea urinary tract obstruction, hemorrhage into GI tract.

Decreased In

Malnutrition, pregnancy, infancy, acromegaly, low protein- High carbohydrate diet, impaired absorption, some patients of nephrotic syndrome.

Blood Urea Nitrogen : 9.5 mg/dL 7.0-18.0

Method : Calculated

Reference

In blood, Urea is usually reported as BUN and expressed in mg/dl. BUN mass units can be converted to urea mass units by multiplying by 2.14. Beckman kit literature.

Note :- Suggested Clinical Correlation *

****End Of Report****

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